



CI Institute of
Nursing

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BRN Case Management CE Package - Draft / Pending BRN CEP Approval

BRN Care Manager / RN Case Management CEP | Draft / Pending BRN CEP Approval

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Quality Assurance, Risk Management, Incident Reporting, and Escalation

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"Pending California Board of Registered Nursing Continuing Education Provider approval. Do not advertise, issue certificates, or represent this course as approved until a BRN CEP number has been issued."

This document follows a CCCCCO-style documentation organization only (structured overview, objectives, content outline, activities, facilitator notes, assessment mapping, compliance support). No CCCCCO content is copied; all content is original and customized for California BRN Case Management CE.

Module At a Glance

Field	Value
Module ID	CM-M08
Module title	Quality Assurance, Risk Management, Incident Reporting, and Escalation
Parent course	RN Case Management in Home Health: Care Coordination, Documentation, Patient Advocacy, and Safe Transitions of Care
Contact hours	3 (draft design math)
Estimated learner minutes	165 (>=150 floor)
Direct/indirect linkage	Indirect patient/client care (quality assurance; legal aspects)
Approval status	Draft / Pending BRN CEP Approval

BRN Relevance Rationale

QA, risk identification, incident reporting, and escalation are nursing patient-safety functions.

Module Description

Develops the RN's ability to recognize quality indicators and risks, complete a compliant incident report, and escalate within a just-culture framework. Highest-scrutiny module - anchored to patient safety, not administration.

Measurable Learning Objectives

By the end of this module the learner will be able to:

- 1 Identify quality indicators relevant to case management.
- 2 Recognize a reportable event and complete a compliant incident report.
- 3 Apply an escalation pathway (who, when, how) to a safety concern.
- 4 Distinguish quality improvement from blame and describe a just-culture response.

Required Readings / Learning Content Sections

- Reading 8.1 - QA fundamentals and indicators.
- Reading 8.2 - Risk identification.
- Reading 8.3 - Incident reporting standards.
- Reading 8.4 - Escalation and just culture.

Detailed Lesson Outline

Lesson ID	Lesson	Est. minutes
CM-M08-L1	QA fundamentals & indicators	35
CM-M08-L2	Risk identification	35
CM-M08-L3	Incident reporting standards	40
CM-M08-L4	Escalation & just culture	35
CM-M08-KC	Knowledge check + incident report	20

Total estimated instructional minutes: **165** (meets the ≥ 150 -minute / 3-contact-hour floor).

Key Terms

Term	Definition
Quality indicator	A measurable element of care used to monitor quality.
Near-miss	An event that could have caused harm but did not.
Incident report	A factual, objective record of an adverse event or near-miss.
Just culture	A framework balancing accountability with learning, not blame.
Escalation pathway	The defined who/when/how for raising a safety concern.

Fictional / De-identified Case Scenario

All clinical content is fictional and de-identified. No PHI. During a fictional home visit, the RN discovers a medication discrepancy that did not reach the patient (a near-miss) caused by an outdated medication list.

Learner Activities

- Read Sections 8.1-8.4.
- Classify the event (near-miss vs. adverse event).
- Complete a de-identified incident report.
- Write an escalation note and complete Knowledge Check 8.

Documentation / Decision-Making Exercise

Complete a de-identified Incident Report for the near-miss and write an escalation note identifying who is notified, when, and how.

Knowledge Check Blueprint

10 items, unlimited attempts, immediate feedback, no full key, $\geq 80\%$ to complete.

Assessment Items (Learner-Facing Stems)

KC8-Q1 (MCQ). On finding a near-miss medication discrepancy, the RN should first:

- A) Ignore it because no harm occurred
- B) Address safety, then file an incident report and escalate
- C) Blame the prior nurse
- D) Delete the medication note

KC8-Q2 (MCQ). An incident report should be:

- A) An opinion about who is at fault
- B) Objective, factual, and timely
- C) Stored only verbally
- D) Written days later from memory

KC8-Q3 (True/False). A just-culture response focuses on system learning rather than individual blame for honest errors.

- True
- False

LMS Completion Criteria

- All lesson pages viewed (activity completion: require view).
- CM-M08 knowledge check passed at $\geq 80\%$ (require passing grade).
- Documentation/decision exercise submitted (require submission).

Evidence Generated by Learner Completion

- Activity-completion timestamps (time-on-task evidence).
- Knowledge-check score and attempt log.
- Submitted exercise artifact (retained ≥ 4 years; no PHI).

Moodle Activity Recommendations

- Page
- Book
- Quiz (KC8)
- Assignment (incident report)

Accessibility Considerations

- Captions + transcripts for any media; semantic headings; alt text on diagrams.
- Sufficient color contrast; no color-only meaning; keyboard-navigable activities.
- Accessible downloadable templates (tagged PDF + DOCX); extended-time accommodation supported.
- Target WCAG 2.1 AA on the built course.

PHI / Privacy Guardrails

- All cases fictional and de-identified; no real patient/resident/family/facility data.
- Compliance review required; incident template uses fictional data with explicit no-PHI banner.
- Submission reminders and (for capstone) attestation + manual PHI review.

SME Review Flags

- [[SME REVIEW REQUIRED]] HIGH SCRUTINY: anchor QA/risk content to patient safety and nursing practice, not pure administration.

Compliance Review Flags

- [[COMPLIANCE REVIEW]] Compliance review required; incident template uses fictional data with explicit no-PHI banner.

Internal Answer Key Appendix (ADMIN / FACULTY ONLY - DO NOT SHOW LEARNERS)

Item ID	Type	Correct answer	Rationale	Remediation
KC8-Q1	MCQ	B	Patient safety first, then objective reporting and escalation.	Review Reading 8.3/8.4.
KC8-Q2	MCQ	B	Reports must be factual, objective, and timely.	Review Reading 8.3.
KC8-Q3	True/False	True	Just culture balances accountability with system learning.	Review Reading 8.4.

Debrief / Remediation Guidance

Connect the near-miss to a system fix (medication-list reconciliation) and a blame-free report. Failed attempts route to Reading 8.3/8.4.

Application-Packet Defensibility Notes

Quality assurance and legal aspects of nursing are expressly BRN-relevant when tied to patient safety. SME must confirm framing.

Program-structure note: This module is part of the 30-contact-hour, 10-module RN case management program (10 modules x 3 contact hours; 50 instructional minutes = 1 contact hour; minimum 1,500 instructional minutes, 1,680 design minutes). The official BRN contact-hour map and transfer worksheet are aligned to this 30-contact-hour structure. Any earlier 4-contact-hour representative course is superseded and is not part of the current BRN CEP signer-review package. [[SME REVIEW REQUIRED BEFORE SUBMISSION]]

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